طبع فى 13:11:51 21/10/2026

جوستاف روسي انترناشيونال - مصر (EPE) *

طلب علاج أول مرة / متردد

إسم المريض رباعياً : امل رياض عبد الخالق حسين

المحافظة : الجيزه **الحي / المدينة :** الشيخ زايد **العنوان :** قصر الباسل مركز اطسا الفيوم

رقم البطاقة : 28901242300401 **رقم التليفون :** 01099544602 **تاريخ تسجيل الإستمارة :** 20-10-2026

العمر(تاريخ الميلاد) : 24-01-1989 **المهنة :** ربه منزل

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:-التأمين الصحي-القوات المسلحه-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

الإخصائي الاجتماعي المقر بما فيه

التوقيع:(----------) **التوقيع:(----------)**

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)

نتائج الأبحاث : (مرفق)

التشخيص النهائي : سرطان بالثدي (مبادرة)

خطة العلاج : أبحاث أورام رنين علي الفقرات القطنيه والعجزيه بالصبغه [12771] **قسم العلاج :** الاورام

التعليق على التشخيص : ----------

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد كيلاني **الوظيفة:** ---------- **التوقيع:** ----------

الاسم: د/ندى حسن **الوظيفة:** ---------- **التوقيع:** ----------

الاسم: د/نرمين رمضان **الوظيفة:** ---------- **التوقيع:** ----------

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية. كما تقر اللجنة بأن البيانات المدونة فى هذا النموذج صحيحة.

تعتمد صحة البيانات المرفقة رئيس اللجنة و مدير المستشفي.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفي قبل الصرف.

أعضاء اللجنة

ا.د/ ---------- **ا.د/** ---------- **د/** ---------- **د/** ----------

قرار اللجنة

التشخيص النهائي : ----------

خطة العلاج : ----------

المستشفي : ----------**قسم :** ----------

جهة العلاج : ----------**نفقات :** ----------

ملاحظات : ----------

رئيس	مدير عام	ختم
المجلس الطبي المتخصص	مديرية الشئون الصحية	شعار الجمهورية

Name: امل رياض عبد الخالق حسين
National ID: 28901242300401

تقرير طبي

السيد

تحية طيبة وبعد,,

إيماء للطلب المقدم من	**بشأن تقرير طبي للمريض**
بقسم:	**رقم طبي:**

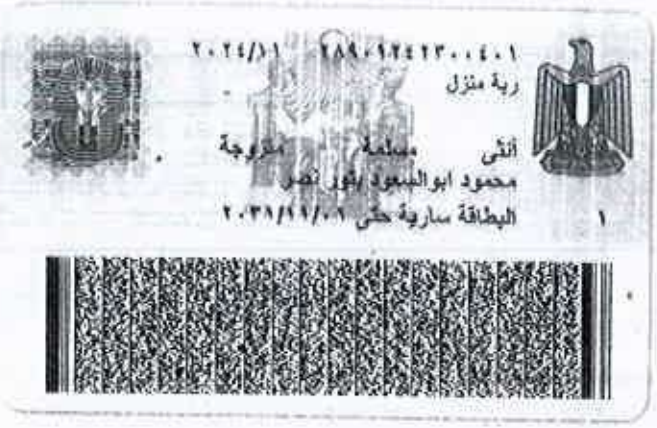
نتشرف بأن نوضح فيما يلي تقريرا عن حالته

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	**تاريخ وساعة الخروج :** PM 01:17 05-07-2026

وبعد الكشف والفحوصات والمتابعة وجد انة

المريضة تعاني من ورم غير حميد بالثدى و تحتاج الي **Contrast-enhanced MRI of the lumbar & Sacral spine**

التاريخ : **05-07-2026**	**الوقت :** **-- --:--**
	الطبيب المعالج : د/شيماء لاشين





National ID – Front & Back
[[DOCTYPE:ID_FRONT_BACK]]

مبادرة رئيس الجمهورية لدعم صحة المرأة المصرية

صحة المصرية هي صحة مصر

MDT FORM

7/9/20

G

Name		MRN	
Age		ID	
Menopausal status		Phone	
NRS SCORE		Marital status	
BMI		Family history	
CO morbidity		REASON OF MDT	

Clinical Data

BX: IDC

ER:	PR:	HER 2:	Ki67:
518	018	2 equiv	

R L

Sonomammography: Denovo met BC to liver. Bm, Brm

TNM

Mets Workup: Pt st.d T-xel, Herceptin

Pt received 36 wk of T-xel & 14 cycl of Herceptin

Pt develop PN/ GIT

Recommendations:

Ust CT CAP 4-2-25 → D.R

MRI br 2-6-2025 → Sols 5 to 14 mm

MDT OUTCOME & PLAN

To Maintain AI + Zoledex (for ER level) + Maintain Herceptin IV

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric

MDT Form
[[DOCTYPE:MDT_FORM]]

Patient Name	: الأستاذه / امل رياض عبد الخالق حسين	Request No.	: 13009702
Patient No.	: 28901242300401	Request Date	: 07/08/2024
Path No.	: SR-1099/24	Verification Date	: 20/08/2024
Sex	: Female	Financial Category	: اورام دار السلام - نفقة الدولة
Age	: 35 Y	Department	: Out Patient
Referreing Doc :		Registered by	: .marwa mohamed youss

Pathology Report

Clinical Data: Right breast mass.
Nature of Specimen: Slide revision.

Gross Examination:
- Five paraffin blocks labeled by (840536) and Nine slides were received.

Microscopic Examination:
* Histological type: Multifocal invasive duct carcinoma.
* Grade (differentiation+ nuclear pleomorphism+ mitosis): Grade II (3+2+1)
* DCIS: Positive (90%) (commedo, cribriform, solid)
* Angioinvasion: No.
* Perineural invasion: No.
* IPT:
- ER: Strong nuclear reaction in 90% of tumor cells (score 8/8).
- PR: No nuclear reaction in the tumor cells (score 0/8).
- Her2: Moderate complete membrane reaction in >10% of tumor cells (score 2+).

Diagnosis:
Right breast cancer, Slide revision:
Multifocal invasive duct carcinoma, Grade II with DCIS.
IPT:
- ER: Positive (score 8/8).
- PR: Negative (score 0/8).
- Her2: Equivocal (score 2+).
- For further IPT (SISH).

Signature

Verified by :
Dr. Sagda El-Kady

غرفة رقم (4) – الدور الأول – مبنى رقم (987) – كورنيش النيل – مصر القديمة – القاهرة – مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Pathology Report
[[DOCTYPE:PATHOLOGY_REPORT]]

Patient Name	: الأستاذه / امل رياض عبد الخالق حسين	Request No.	: 13009702
Patient No.	: 28901242300401	Request Date	: 07/08/2024
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Signature

Verified by :
Dr. Sagda El-Kady

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Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Pathology Report
[[DOCTYPE:PATHOLOGY_REPORT]]

Patient Name: Amaal Reyad Abd Al Khalek Hussein	Date : 10/05/2025
Exam : C.T CHEST WITH CONTRAST	Age : 36 Year(s)
Medical ID: 2509/2024	Pat. ID: 77262

POST -CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN & PELVIS REVEALED: -

- **Known case of right BCS . **As compared to the last study 23-01-2025, The current revealed:**

1. ***Chest:***

- Right breast still shows rather stationary appearance of the UIQ ill-defined parenchymal asymmetry with diffuse skin thickening
- **Stable course** regarding to the size of the right axillary level I, II and III enlarged lymph nodes became prominent the largest measures about 1x0.5 cm with irregular outlines and ill-defined axillary sheets.
- Non appreciated forenoted Prominent right internal mammary lymph nodes.
- **Stationary** course regarding to size of Few bilateral scattered tiny mostly subpleural / pleural based pulmonary nodules, the largest is noted at inferior lingular segment still measuring 4 mm.
- **Still noted unchanged** bilateral apical pleuropulmonary scarring.
- No pathological hilar or mediastinal lymph nodes.
- No evidence of pleural effusion, pleural thickening or masses seen.
- Average cardiac size. No pericardial effusion detected.
- Normal CT appearance of the thoracic aorta and the great vessels.

2. ***Abdomen and pelvis:***

- The liver is of average size with ***rather stationary course regarding number and size of*** bilobar discrete and confluent hypodense focal lesions, The largest at hepatic dome measuring currently about 1.3 x1 cm.
- Patent homogenous portal vein and its main tributaries
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- No pathological abdominal or pelvic lymph nodes.

CT Scan
[[DOCTYPE:CT_SCAN]]

Patient Name: Amaal Reyad Abd Al Khalek Hussein	Date : 22/07/2025
Exam : CT CAP+C (CT Chest Abd. Pelvis)	Age : 36 Year(s)
Medical ID:	Pat. ID: 77262

- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- No pathological abdominal or pelvic lymph nodes.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas, aorta, I.V.C and both supra-renals.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- Currently noted bilateral adnexal cystic lesions for sonographic assessment.
- Normal appearance of the other pelvic organs.
- No abdominal or pelvic masses or collections.

3. ***The scanned skeleton:***

- Sacralized L5 vertebra. Stationary course of D4, L1 vertebral body and left iliac wing lytic lesions measuring 7 mm. Small D5 sclerotic lesion.1st degree lytic spondylolisthesis of L4.
- No vertebral structural collapse or extra-osseous soft tissue component.

OPINION:

Reported case of right BCS SHOWING

- **Overall Regressive course since the last study.**

Dr/Aya Gabr.

CT Scan
[[DOCTYPE:CT_SCAN]]

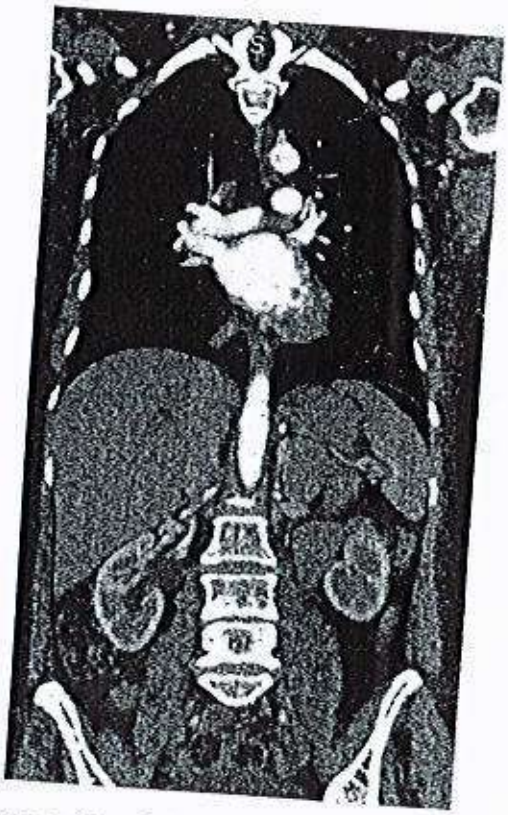
Patient Name: Amaal Reyad Abd Al Khalek Hussein	Date : 22/07/2025
Exam : CT CAP+C (CT Chest Abd. Pelvis)	Age : 36 Year(s)
Medical ID:	Pat. ID: 77262

POST -CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN & PELVIS REVEALED: -

- **Known case of right BCS . **As compared to the last study 10-05-2025, The current revealed:**

1. ***Chest:***

- Right breast still shows rather stationary appearance of the UIQ ill-defined parenchymal asymmetry with diffuse skin thickening
- **Regressive course** regarding to the size of the right axillary level I, II and III enlarged lymph nodes became prominent the largest measures about 0.5x0.3 cm with irregular outlines and ill-defined axillary sheets.
- Non appreciated forenoted Prominent right internal mammary lymph nodes.
- **Regressive** course regarding to size of Few bilateral scattered tiny mostly subpleural / pleural based pulmonary nodules, seen less appreciated in the current study.
- **Still noted unchanged** bilateral apical pleuropulmonary scarring.
- No pathological hilar or mediastinal lymph nodes.
- No evidence of pleural effusion, pleural thickening or masses seen.
- Average cardiac size. No pericardial effusion detected.
- Normal CT appearance of the thoracic aorta and the great vessels.



2. ***Abdomen and pelvis:***

- The liver is of average size with ***Regressive course regarding number and size of*** bilobar discrete hypodense focal lesions, seen less appreciated in the current study
- Patent homogenous portal vein and its main tributaries
- No intra-hepatic or extra-hepatic biliary radicles dilatation.

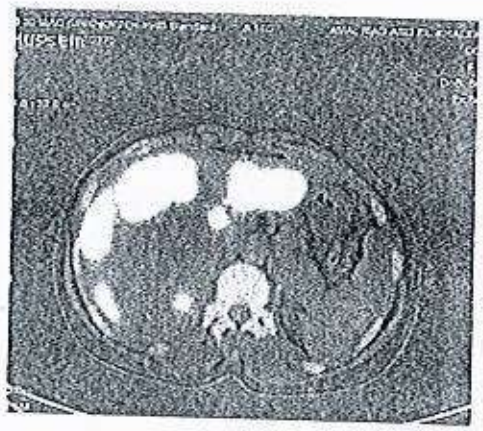
CT Scan
[[DOCTYPE:CT_SCAN]]

Patient Name:	*Amal Riad Abd El-Khalek Hussein*	*Patient ID:*	*684364*
Prof. Dr:	*Clinics Masr El-Hadetha*	*Exam Date:*	*3/9/2024*
Accession:	*240903134*		

III) ***Chest:-***

- No metabolically active (FDG avid) pulmonary nodules or masses are noted.
- No metabolically active (FDG avid) pleural effusion is noted.
- No FDG avid (metabolically active) mediastinal lymph-nodes are noted.
- Normal cardiac size with no pericardial effusion.

IV) ***Abdomen and pelvis:-***



- Mildly enlarged liver showing few metabolically active ill-defined bilobar hypodense hepatic focal lesions measuring up to 7 x 5.6 cm with max SUV ~ 12.77 (at hepatic dome lesion).
- Metabolically active few enlarged aortocaval, portocaval and porta hepatis lymph nodes are noted measuring up to 3 cm with max SUV ~ 10.45 (at porta hepatis hepatic lymph node).
- Average sized spleen showing no metabolically active (FDG avid) focal lesions.
- No ascites.
- Metabolically active right adnexal cyst is noted measuring 2.7 cm with max SUV ~ 7.18 (corpus luteum).
- The rest of the abdominal and pelvic organs show normal PET/CT appearance with no related focal lesions or abnormal FDG uptake.

CT Scan
[[DOCTYPE:CT_SCAN]]

GUSTAVE ROUSSY INTERNATIONAL EGYPT

Name : Amal Reyad abdel khalek hussei	**Date : 10-02-2026 10:56**
Branch : Portable	**Medical No. : GRI-77262**
Ref.Physician Prof : CT CAP+C	**Ins.Company :**

POST -CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN & PELVIS REVEALED: -

- **Known case of right BCS . Compared to the last study 18/11/2025, The current revealed:**

1. **Chest:**

- **No appreciable time interval changes** regarding to the size of the few right axillary lymph nodes became prominent the largest measures about 1x0.5 cm.
- Rather stationary course regarding the previously seen few tiny bilateral scattered pulmonary nodules largest measuring 3.5 mm in diameter at the left lower lobe.
- **No appreciable time interval changes regarding the number of the previously seen few bilateral scattered faint ground glass densities.**
- **Still noted unchanged** bilateral apical pleuropulmonary scarring.
- No pathological hilar or mediastinal lymph nodes.
- No evidence of pleural effusion, pleural thickening or masses seen.
- Average cardiac size. No pericardial effusion detected.
- Normal CT appearance of the thoracic aorta and the great vessels.

2. **Abdomen and pelvis:**

- The liver is of average size ***with regressive course regarding the previously seen few bilobar*** hypodense focal lesions, The largest at segment VIII measuring currently about 6 mm in diameter in today's study.
- Patent homogenous portal vein and its main tributaries
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- No pathological abdominal or pelvic lymph nodes.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas, aorta, I.V.C and both supra-renals.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- Regressive course mounting to complete resolution regarding the previously seen right ovarian cyst.
- Normal appearance of the other pelvic organs.
- Regressive course mounting to complete resolution regarding the previously seen mild pelvic collection.

3. *The scanned skeleton:*

CT Scan
[[DOCTYPE:CT_SCAN]]

GUSTAVE ROUSSY
INTERNATIONAL
EGYPT

Name : Amal Reyad abdel khalek hussei	**Date : 10-02-2026 10:56**
Branch : Portable	**Medical No. : GRI-77262**
Ref.Physician Prof : CT CAP+C	**Ins.Company :**

- No appreciable time interval changes regarding the previously seen few vertebral and left iliac wing lytic as well as the D5 sclerotic focus.
- Still seen 1st degree lytic spondylolisthesis of L4.
- No vertebral structural collapse or extra-osseous soft tissue component.

OPINION:

Reported case of right BCS SHOWING

- **Regressive course regarding the:**
  - **The size and number of the previously seen few bilobar hypodense hepatic focal lesions.**
  - **The previously seen right ovarian cyst no longer seen in today's study.**
  - **The previously seen mild pelvic collection.**
- **Stationary course regarding the rest of the study.**

Dr. Sherif Wagdy

Dr. Sherif Wagdy

CT Scan
[[DOCTYPE:CT_SCAN]]

Name : Amaal ryad abd elkhalik hussin	**Date : 10-02-2026 11:20**
Branch : Portable	**Medical No. : GRI-260210111719STD131221107523748902**
Ref.Physician Prof : brain +c	**Ins.Company :**

MRI OF THE BRAIN

Technique :
Axial T1, T2 ,FLAIR, DWI& ADC
Coronal T2
Sagittal T2
Postcontrast T1

Findings: ***compared to last study dated 21-6-2025 ;***

- Marked regressive course as regard the size and shape of the previously noted multiple bilateral cerebral SOL seen at both frontal , parietal basal ganglia and , both thalami and right subependymal and left cerebellar region , the lesion currently seen as foci and small areas of altered MR signal eliciting low T1 and bright T2 and FLAIR WI signal with contrast uptake. some of them show minimal surrounding edema. no significant mass effect ,the largest one is seen at right lateral ventricle sub ependymal location measuring about 6mm compared to larest one 14 mm in the las study with regression of the previously noted marked perifocal edema .
- Normal size and configuration of the supra and infra tentorial ventricular system
- No midline shift or deformity
- Normal cortical sulci
- No cerebello pontine angle masses.
- Partial empty sella.
- Prominent peri-optic CSF spaces bilaterally.

OPINION:

- ***Marked regressive course of the previously noted cerebral and cerebellar intra axila SOLs compared to last study , for follow up and clinical correlation.***

M.R.I
[[DOCTYPE:MRI]]